PHARMACOVIGILANCE INTERNSHIP

ASSIGNMENT 02 - ADR REPORTING & DOCUMENTATION

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Date: 07-11-2026

1. OBJECTIVES

To document a suspected adverse drug reaction using a structured ADR reporting format. The report records the patient details, suspected medicine, reaction chronology, relevant history, seriousness, outcome and reporter information while distinguishing documented facts from information that was not supplied.

2. CASE NARRATIVE

A 32-year-old male patient weighing 65 kg was diagnosed with acute bacterial sinusitis. He was prescribed oral amoxicillin 400 mg three times daily for five days, starting on 01 March 2026. On 03 March 2026, he developed generalized itching and skin rashes over the chest. Amoxicillin was stopped on the same day and antihistamines were administered. The patient recovered completely by 06 March 2026.

The case narrative also states that the patient had no history of allergy. It mentions no family history of diabetes or hypertension. These family-history details are reproduced as provided; they do not establish the patient's own diabetes or hypertension status.

3. COMPLETED ADR REPORTING FORM

The following is an educational transcription of the information available in the case. Unknown fields are deliberately marked “Not provided”; no patient, product or reporter details have been invented.

3.1 Patient Information

Field	Documented information
Patient initials	RP (initials only to protect confidentiality)
Age	32 years
Sex	Male
Weight	65 kg
Patient name / address	Not recorded; maintain patient confidentiality

3.2 Suspected Adverse Reaction

Field	Documented information
Date reaction started	03 March 2026
Date of recovery	06 March 2026
Description of reaction	Generalized itching (pruritus) and skin rashes over the chest after starting amoxicillin.
Suspected reaction	Pruritus and rash; suspected drug-associated cutaneous reaction. This is not a confirmed allergy diagnosis.

3.3 Suspected Medication

Field	Documented information
Suspected medicine	Amoxicillin (generic name)
Dose	400 mg
Route	Oral
Frequency	Three times daily (TID)
Therapy start date	01 March 2026
Therapy stop date	03 March 2026
Indication	Acute bacterial sinusitis
Manufacturer / brand	Not provided in the case narrative
Batch number / expiry date	Not provided in the case narrative
Dechallenge	Yes. The medicine was stopped on 03 March 2026; recovery was reported by 06 March 2026.
Rechallenge	Not done / not applicable. Re-exposure is not reported.

3.4 Concomitant Medication and Treatment of Reaction

Field	Documented information
Other medicines before / during reaction	None reported in the case narrative.
Treatment for the reaction	Antihistamines; specific medicine, dose, route and dates are not provided.

Documentation note	Antihistamines are treatment for the reaction, not pre-existing concomitant medication.

3.5 Relevant Tests and Laboratory Data

Not provided in the case narrative. No laboratory results or investigations should be inferred.

3.6 Relevant Medical / Medication History

Field	Documented information
Personal allergy history	No history of allergy reported.
Diabetes / hypertension	The narrative reports no family history of diabetes or hypertension; the patient's own history is not specified.
Pregnancy	Not applicable for this male patient.
Smoking / alcohol	Not provided.
Hepatic / renal dysfunction	Not provided.
Other medical / medication history	Not provided.

3.7 Seriousness Assessment

Classification based on the supplied narrative: NON-SERIOUS.

No death, life-threatening event, initial or prolonged hospitalization, persistent/significant disability, congenital anomaly, or intervention to prevent permanent impairment or damage is reported.

The described itching and chest rash resolved after withdrawal of the suspected medicine and antihistamine treatment. This classification is limited to the facts supplied; additional clinical information could change it.

3.8 Outcome

Recovered / resolved — complete recovery was reported by 06 March 2026.

3.9 Reporter Details

Field	Documented information
Reporter name	ANISHA HUSSAIN
Professional / institutional address	VirtualWorks

PIN code	-
Email address	-
Contact number	-
Occupation	Pharmacovigilance Intern
Date of report	07-11-26
Causality assessment	To be completed separately in Assignment 03, using the method instructed by VirtualWorks.

4. CHRONOLOGICAL TIMELINE

Date	Event documented
01 March 2026	Amoxicillin 400 mg orally three times daily initiated for acute bacterial sinusitis; planned duration stated as five days.
03 March 2026	Generalized itching and chest rash developed. Amoxicillin stopped on the same day. Antihistamines given.
06 March 2026	Complete recovery reported.

5. PRELIMINARY PHARMACOVIGILANCE INTERPRETATION

The reaction occurred after amoxicillin was started, and recovery followed withdrawal of the medicine. This temporal sequence and positive dechallenge support a possible relationship. However, the available narrative does not provide enough information to establish causality conclusively. Rechallenge was not reported, and alternative causes or examination findings are not described. Formal causality scoring/classification is reserved for Assignment 03.

The tutorial describes the example as a Type B (bizarre / unpredictable), non-dose-dependent reaction. This is recorded as the tutorial's teaching point, not as a definitive independent clinical conclusion from the limited case data.

6. DATA-QUALITY NOTE: DOSE DISCREPANCY

The original case narrative states amoxicillin 400 mg three times daily, but during the demonstration of the form the presenter later says 500 mg. This report uses 400 mg because it is the dose stated in the original case scenario. The discrepancy should be clarified with the training provider if an official case sheet gives a different dose.

The manufacturer/brand, batch number, expiry date, exact antihistamine, its dose and route, laboratory results, rechallenge status beyond “not reported”, and reporter's contact details were not supplied. They have not been fabricated.

7. CONCLUSION

This educational case documents a suspected non-serious cutaneous adverse reaction consisting of generalized itching and chest rash after amoxicillin therapy. Amoxicillin was withdrawn, antihistamines were administered, and complete recovery was reported by 06 March 2026. The available information has been organized into a structured ADR report, with missing details and the dose inconsistency transparently identified.

9. REFERENCES

1. “How to fill ADR form? Step by Step Guide.” YouTube tutorial supplied for VirtualWorks Task 2

2. Pharmacovigilance Programme of India (PvPI), Indian Pharmacopoeia Commission. Suspected Adverse Drug Reaction Reporting Form and related reporting guidance. Use the current official form and instructions for real-world reporting.